AR 2EB

ALAMEDA RESEARCH
2: ELECTRIC BOOGALOO
LONG HORIZONS · STRUCTURAL SHIFTS · IMAGINATION

INTERNAL RESEARCH · MEMO · NOT INVESTMENT ADVICE · AI-ASSISTED

GRAIL

NASDAQ : GRAL · Young-Company DCF (Damodaran)

$5.83B mkt cap · 44M sh · $823M cash, zero debt · $0.823B cash (Q1'26), no debt; ~44M shares (low float); Illumina ~14.5% at the 2024 spin; Revenue-generating: $147M FY25 (+17%), Galleri self-pay ~$949; burn ~$300M/yr (~2.5yr runway); NHS-Galleri missed primary endpoint (Feb'26); FDA PMA accepted; Medicare law (≥2028, ~$509 parity); Abbott bought Exact

22 October 2026 · the qualitative

$132.60

THE CENTRAL QUESTION

Does ~$4.8B / ~32x sales price GRAIL's multi-gate regulatory option fairly — after the NHS-Galleri miss, but with the FDA filing accepted and the Medicare law in hand?

GRAIL trades at ~$4.8B (≈$108/sh on just ~44M shares) on ~$147M FY25 revenue and ~$823M cash. Galleri — a blood test for 50+ cancers, self-pay today — is a multi-gate option: FDA PMA (filed Jan 2026, now accepted), then Medicare coverage (signed Feb 2026; ≥2028 at ~$509 parity), then broad adoption. The pivotal NHS-Galleri trial MISSED its primary endpoint (-50% stock), but the PMA rests on positive PATHFINDER 2 and the law passed — broken yet de-risked. Re-modeled on ~$509 parity (Abbott now backs Cancerguard), at ~$108 the market prices well above the modal regulatory-success case — it needs the broad-adoption upside.

PROBABILITY-WEIGHTED EXPECTED VALUE

most likely case (base) -56.0% · 82% at or below spot

$65.99 expected fair value today

-50.2% vs spot $132.60

FORWARD COMPOUNDED VALUE

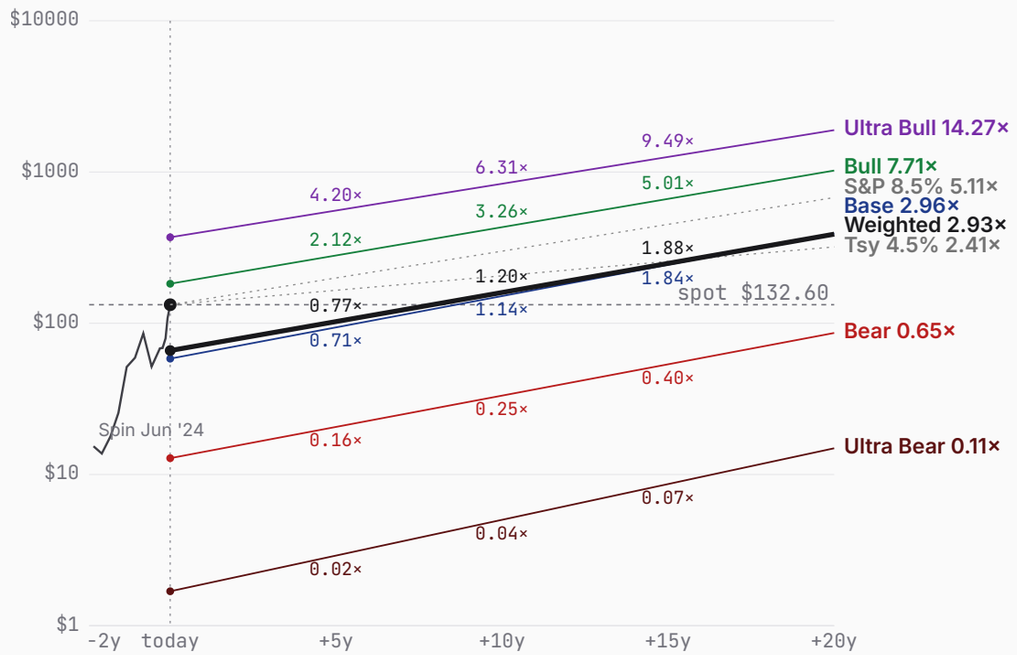
+5y	+10y	+15y	+20y
$102.64	$159.77	$248.91	$388.13
0.77× spot	1.20× spot	1.88× spot	2.93× spot

WEIGHTING RATIONALE

Ultra Bear 20% FDA rejects; no coverage; near-total loss.
Bear 28% FDA late; Medicare narrow; sub-scale; low teens.
Base 34% FDA 2027 + Medicare 2028; leader; -46%.
Bull 13% Broad adoption; category lead; ~1.7×.
Ultra Bull 5% MCED standard of care; dominant; >3×.

Bull (13%) + Ultra Bull (5%) together contribute $42.23 — 64% of the $65.99 expected value despite 18% combined probability. Today's spot ($132.60) sits 101% above the weighted expected; the forward-weighted value reaches spot in about 8 years.

PRICE + PROJECTIONS · HISTORICAL THROUGH PROBABILITY-WEIGHTED FORWARD



ULTRA BEAR — $1.69

-98.7% vs spot

TAM 1.3% P(fail) 40%
S2C 1.0 Prob 20%

FDA rejects; no coverage; self-pay niche.

BEAR — $12.82

-90.3% vs spot

TAM 4.1% P(fail) 22%
S2C 1.1 Prob 28%

FDA late; Medicare narrow; modest volume.

BASE — $58.34

-56.0% vs spot

TAM 8.2% P(fail) 12%
S2C 1.3 Prob 34%

FDA 2027 + Medicare 2028; category leader.

BULL — $182.48

+37.6% vs spot

TAM 14.4% P(fail) 6%
S2C 1.5 Prob 13%

Broad adoption; annual-repeat screening.

ULTRA BULL — $370.13

+179.1% vs spot

TAM 22.4% P(fail) 3%
S2C 1.7 Prob 5%

MCED standard of care; GRAIL dominant.

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GRAIL scenario narratives

PROBABILITY WEIGHTS

Ultra Bear 20% Bear 28% Base 34% Bull 13% Ultra Bull 5% · Weighted expected $65.99 (-50.2% vs spot)

ULTRA BEAR $1.69

-98.7% vs spot

Probability 20%

FDA rejects; no coverage; self-pay niche.

WHY 20%

The NHS-Galleri miss is a real, fired datapoint — the only RCT of any MCED failed its stage-shift primary endpoint, hardening FDA and payer skepticism. 40% within-scenario failure reflects genuine PMA-rejection risk. 20% scenario weight.

WHAT HAPPENS

The FDA PMA is rejected or stalled — the NHS-Galleri primary-endpoint miss (no significant stage III+IV reduction) hardens FDA/USPSTF skepticism about clinical utility, and Galleri stays a self-pay LDT niche. Test volume plateaus, the Medicare benefit never activates (it is gated on FDA approval), and GRAIL burns its ~$823M cash defending the franchise.

With a 40% within-scenario failure probability and revenue stalled near $0.3B, the equity collapses toward the cash-and-distress floor — a near-total loss vs today's $108.

BEAR $12.82

-90.3% vs spot

Probability 28%

FDA late; Medicare narrow; modest volume.

WHY 28%

FDA approves but adoption disappoints — Medicare narrow, private payers await mortality proof, ASP compresses. 22% failure probability. 28% weight as a very plausible 'approved but sub-scale' outcome.

WHAT HAPPENS

The FDA approves but late, and Medicare coverage arrives narrow and slow; private payers wait for the mortality data the NHS trial did not deliver. Galleri reaches modest reimbursed volume (~$1B revenue by 2035) at thin-but-improving margins — a real test, sub-scale economics.

The DCF lands in the low teens — well below spot. This is the 'approved, but adoption disappoints' world the NHS miss makes plausible.

BASE $58.34

-56.0% vs spot

Probability 34%

FDA 2027 + Medicare 2028; category leader.

WHY 34%

Requires FDA approval (on PATHFINDER 2, which was positive) + Medicare activation (the law is already signed) + broad coverage — the machinery is in motion. 12% failure probability. 34% weight as the central, modal outcome.

WHAT HAPPENS

The modal path: the FDA approves Galleri (~2027) on the PATHFINDER 2 evidence, Medicare coverage activates (~2028), and broad private coverage follows — Galleri becomes the leading reimbursed MCED test at ~$2.05B revenue by 2035 (~8% of a ~$25B screening market) at ~30% operating margins, blended ASP settling toward the ~$509 Medicare parity rate.

DCF ~$65/share — well below today's ~$108. The market prices above this modal regulatory-success case; the small ~50M share count makes the per-share value swing hard on each gate.

BULL $182.48

+37.6% vs spot

Probability 13%

Broad adoption; annual-repeat screening.

WHY 13%

A chain: clean FDA + Medicare + broad private coverage + annual-repeat adoption + category leadership. Each plausible at 50-70%; jointly ~13%. GRAIL's breadth and only-RCT dataset are the edge.

WHAT HAPPENS

The bull: FDA + Medicare clear cleanly, broad private coverage and annual-repeat screening take hold, and GRAIL extends its first-mover, broadest-test, only-RCT-dataset lead — ~$3.6B revenue by 2035 at ~36% operating margins. The screening category compounds and GRAIL leads it even as ASP holds near parity.

DCF ~$193/share — ~1.8× spot. This is the scenario the regulatory machinery (PMA accepted + Medicare law) keeps alive despite the NHS miss.

ULTRA BULL $370.13

+179.1% vs spot

Probability 5%

MCED standard of care; GRAIL dominant.

WHY 5%

Tail of tails: MCED standard of care + eventual mortality proof + GRAIL dominance + premium terminal. Individually 40-60%; jointly ~5%. The asymmetric upside the ~43M share count amplifies.

WHAT HAPPENS

The tail: MCED becomes standard of care, the extended NHS follow-up eventually shows a mortality benefit, and GRAIL is the dominant platform — ~$5.6B revenue by 2035 at ~40% margins with a premium terminal.

DCF $381/share — 3.5× spot. The asymmetric upside the low share count amplifies; individually each gate is plausible, jointly ~5%.

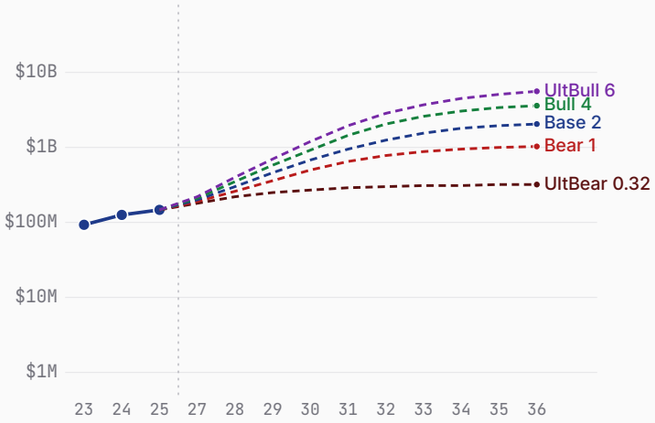
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GRAIL business snapshot

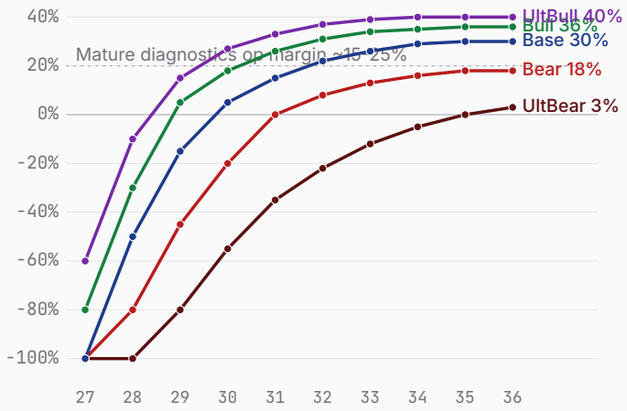
Bear $12.82 Base $58.34 Bull $182.48

FY26-FY35 scenario projections (revenue-generating; pre-profit) · fiscal years end Dec 31 · Q1 2026 10-Q, FY25 results

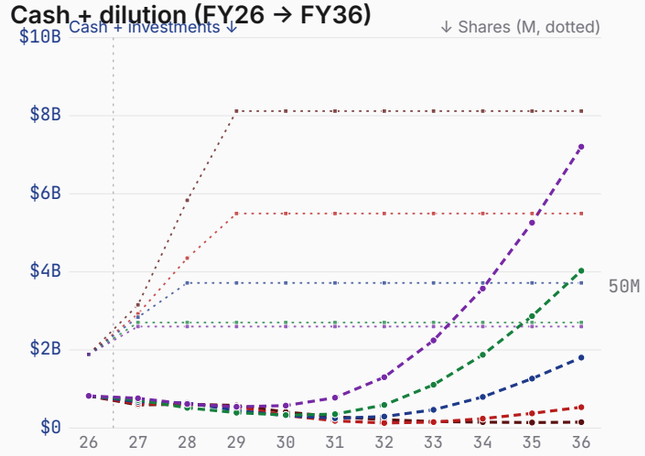
Revenue — history + scenarios to FY36 (log)



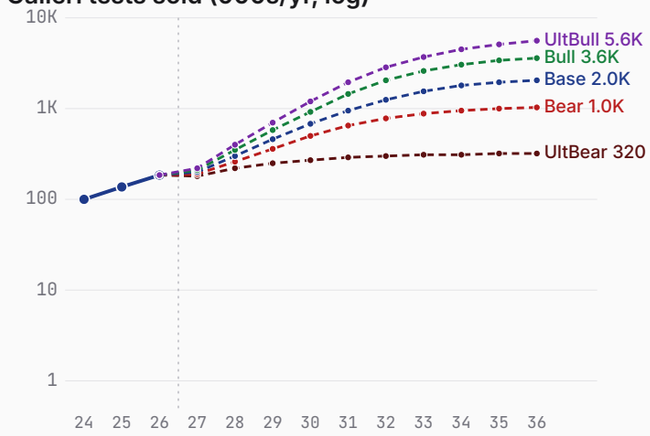
Path to profitability — op margin (FY27→FY36)



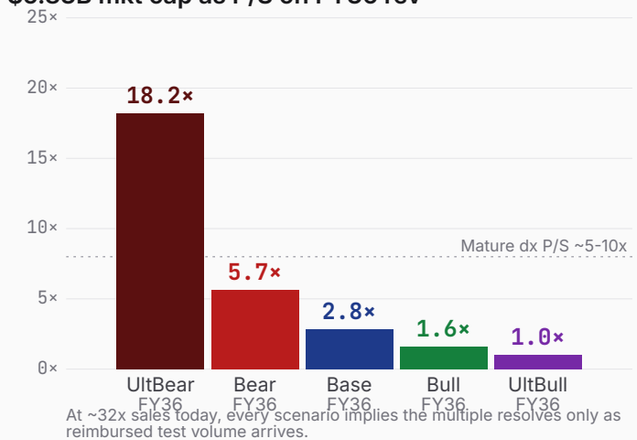
Cash + dilution (FY26 → FY36)



Galleri tests sold (000s/yr, log)

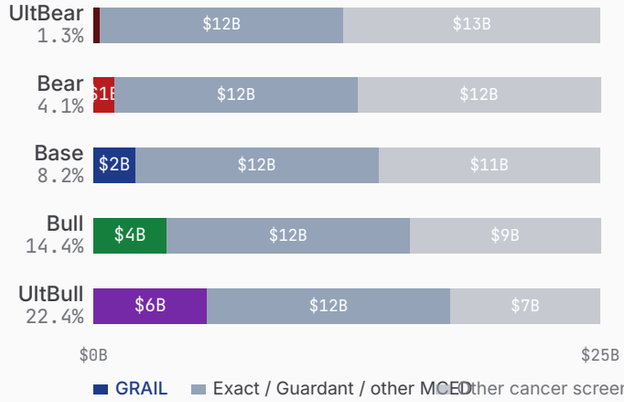


$5.83B mkt cap as P/S on FY36 rev



At ~32x sales today, every scenario implies the multiple resolves only as reimbursed test volume arrives.

$25B FY35 MCED screening TAM — GRAIL share



Sources: GRAIL Q1 2026 10-Q (cash $0.823B), FY2025 results ($147M revenue, net loss), NHS-Galleri (primary endpoint missed Feb'26) + PATHFINDER 2 (positive) readouts, FDA PMA (accepted), Medicare MCED Coverage Act (≥2028, ~$509 parity). Revenue modeled as reimbursed test volume x ASP compressing toward ~$509; FCF NOL-shielded pre-tax (documented simplification). TAM: MCED screening slice ~$25B by FY35 (GRAIL low-teens share).

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GRAIL 7 Powers · the moat, scored

Bear $12.82 Base $58.34 Bull $182.48

Arena. Multi-cancer early detection (MCED) blood screening — GRAIL (Galleri, 50+ cancers, only RCT) vs Abbott/Exact (Cancerguard + Cologuard cash engine), Guardant (Shield, FDA-approved CRC → MCED), Freenome, Delfi; the race to originate the first FDA-approved, reimbursed MCED franchise.

POWER ORIGINATION
origination window: open

POWER ORIGINATION · 7 POWERS

Scale Economies	A reimbursed-volume cost curve is future; today's self-pay volume is sub-scale.
Network Economies	No network flywheel yet; data scale is the closest analog.
Counter-Positioning	Galleri's 50-cancer breadth + build-own dataset is hard for single-cancer (Shield CRC) entrants to mirror without their own large trials.
Switching Costs	Provider-ordering and EHR integration build mild lock-in; early.
Branding	The MCED category-definer brand; strong but dented by the NHS miss.
Cornered Resource · thesis leans here	The only randomized MCED dataset (NHS ~140k) + first PMA + largest commercial test base — the strongest, if contested, origination asset.
Process Power	Assay + bioinformatics pipeline is real but not a demonstrated durable process edge.

THE RACE · NAMED RIVALS

Abbott / Exact Sciences — Abbott-backed; global scale

~27% terminal share · Cancerguard MCED (LDT, launched Sep'25, $689) now owned by Abbott (deal closed Mar'26) — a 160-country distributor behind the Cologuard franchise.

Guardant Health — ~$700M-rev co.

~22% terminal share · Shield FDA-approved for CRC (Medicare $1,495); extending to MCED (Breakthrough Device).

Freenome — $1.35B private

~10% terminal share · CRC-first → multi-cancer; ~$1.35B raised (Roche, T. Rowe).

Delfi Diagnostics — Merck GHI + rounds

~5% terminal share · FirstLook Lung (fragmentomics); niche, lung-first.

LEAD / LAG · FALSIFIERS

MCED clinical evidence (RCT)	NHS ~140k (only RCT; primary missed) vs none randomized	LEADING
FDA approval	PMA accepted (decision 2026-27) vs Guardant Shield approved (CRC)	ROUGHLY LEVEL
Reimbursement / commercial scale	self-pay; Medicare gated to 2028 vs Exact Cologuard reimbursed engine	LAGGING

COMPETITIVE READ

GRAIL is originating the MCED category — the broadest test (50+ cancers), the only randomized dataset, and the first PMA — but the NHS-Galleri primary-endpoint miss dented the 'proven population benefit' narrative, and Abbott/Exact (Cancerguard on a 160-country distributor) and Guardant (FDA + Medicare in CRC) are scaling MCED behind it. The origination window is open but closing: GRAIL must convert the PMA into approval + Medicare coverage before better-capitalized rivals scale, and the extended NHS mortality follow-up is the falsifier that could reopen — or close — the whole thesis.

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GRAIL show your work

Bear $12.82 Base $58.34 Bull $182.48 · Weighted $65.99 (-50.2%)

ULTRA BEAR $1.69

ASSUMPTIONS

TAM Year-10 ($B)	25
Mature share (%)	1.3
Mature op margin (%)	3
Sales-to-capital	1.0
Terminal WACC (%)	11.5
Terminal growth (%)	2.0
P(failure) (%)	40
Scenario probability (%)	20

10-YEAR DCF · $B

FY	Rev	Margin	FCF	PV FCF
FY27	0.18	-140%	-0.28	-0.25
FY28	0.22	-110%	-0.28	-0.21
FY29	0.25	-80%	-0.23	-0.15
FY30	0.27	-55%	-0.17	-0.10
FY31	0.29	-35%	-0.12	-0.06
FY32	0.30	-22%	-0.08	-0.04
FY33	0.31	-12%	-0.05	-0.02
FY34	0.31	-5%	-0.01	-0.01
FY35	0.32	+0%	-0.01	-0.00
FY36	0.32	+3%	+0.01	+0.00

Σ PV explicit FCF	**-0.84**
+ PV terminal (g 2.0%)	0.03

EQUITY BUILD · $B & PER SHARE

Operating EV	$-0.81B
+ Cash & investments	$0.82B
= Equity value	**$0.01B**
Raised over period	$0.70B
Dilution by FY36	+50%
FY36 shares (M)	66
DCF per share	**$0.15**
× (1−P_fail) [60%]	$0.09
+ P_fail × distress [40% × $4.00]	$1.60

= Expected per share $1.69

FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT

+5y	+10y	+15y	+20y
$2.91	$5.02	$8.65	$14.91
0.02×	0.04×	0.07×	0.11×

BEAR $12.82

ASSUMPTIONS

TAM Year-10 ($B)	25
Mature share (%)	4.1
Mature op margin (%)	18
Sales-to-capital	1.1
Terminal WACC (%)	10.0
Terminal growth (%)	2.5
P(failure) (%)	22
Scenario probability (%)	28

10-YEAR DCF · $B

FY	Rev	Margin	FCF	PV FCF
FY27	0.19	-120%	-0.27	-0.23
FY28	0.26	-80%	-0.27	-0.21
FY29	0.36	-45%	-0.25	-0.17
FY30	0.50	-20%	-0.23	-0.14
FY31	0.65	+0%	-0.14	-0.07
FY32	0.78	+8%	-0.06	-0.03
FY33	0.88	+13%	+0.02	+0.01
FY34	0.95	+16%	+0.09	+0.03
FY35	1.00	+18%	+0.14	+0.05
FY36	1.03	+18%	+0.16	+0.05

Σ PV explicit FCF	**-0.71**
+ PV terminal (g 2.5%)	0.70

EQUITY BUILD · $B & PER SHARE

Operating EV	$-0.01B
+ Cash & investments	$0.82B
= Equity value	**$0.81B**
Raised over period	$0.65B
Dilution by FY36	+27%
FY36 shares (M)	56
DCF per share	**$14.46**
× (1−P_fail) [78%]	$11.28
+ P_fail × distress [22% × $7.00]	$1.54

= Expected per share $12.82

FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT

+5y	+10y	+15y	+20y
$20.65	$33.25	$53.55	$86.25
0.16×	0.25×	0.40×	0.65×

BASE $58.34

ASSUMPTIONS

TAM Year-10 ($B)	25
Mature share (%)	8.2
Mature op margin (%)	30
Sales-to-capital	1.3
Terminal WACC (%)	10.0
Terminal growth (%)	2.5
P(failure) (%)	12
Scenario probability (%)	34

10-YEAR DCF · $B

FY	Rev	Margin	FCF	PV FCF
FY27	0.20	-100%	-0.24	-0.21
FY28	0.30	-50%	-0.23	-0.18
FY29	0.46	-15%	-0.19	-0.13
FY30	0.68	+5%	-0.14	-0.08
FY31	0.95	+15%	-0.07	-0.04
FY32	1.25	+22%	+0.04	+0.02
FY33	1.55	+26%	+0.17	+0.08
FY34	1.80	+29%	+0.33	+0.14
FY35	1.95	+30%	+0.47	+0.17
FY36	2.05	+30%	+0.54	+0.18

Σ PV explicit FCF	**-0.06**
+ PV terminal (g 2.5%)	2.48

EQUITY BUILD · $B & PER SHARE

Operating EV	$2.42B
+ Cash & investments	$0.82B
= Equity value	**$3.24B**
Raised over period	$0.40B
Dilution by FY36	+14%
FY36 shares (M)	50
DCF per share	**$64.80**
× (1−P_fail) [88%]	$57.02
+ P_fail × distress [12% × $11.00]	$1.32

= Expected per share $58.34

FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT

+5y	+10y	+15y	+20y
$93.96	$151.32	$243.70	$392.48
0.71×	1.14×	1.84×	2.96×

BULL $182.48

ASSUMPTIONS

TAM Year-10 ($B)	25
Mature share (%)	14.4
Mature op margin (%)	36
Sales-to-capital	1.5
Terminal WACC (%)	9.0
Terminal growth (%)	3.0
P(failure) (%)	6
Scenario probability (%)	13

10-YEAR DCF · $B

FY	Rev	Margin	FCF	PV FCF
FY27	0.21	-80%	-0.21	-0.19
FY28	0.35	-30%	-0.20	-0.16
FY29	0.58	+5%	-0.12	-0.09
FY30	0.92	+18%	-0.06	-0.04
FY31	1.45	+26%	+0.02	+0.01
FY32	2.05	+31%	+0.24	+0.12
FY33	2.60	+34%	+0.52	+0.24
FY34	3.05	+35%	+0.77	+0.33
FY35	3.40	+36%	+0.99	+0.39
FY36	3.60	+36%	+1.16	+0.42

Σ PV explicit FCF	**+1.05**
+ PV terminal (g 3.0%)	7.20

EQUITY BUILD · $B & PER SHARE

Operating EV	$8.25B
+ Cash & investments	$0.82B
= Equity value	**$9.07B**
Raised over period	$0.20B
Dilution by FY36	+7%
FY36 shares (M)	47
DCF per share	**$192.98**
× (1−P_fail) [94%]	$181.40
+ P_fail × distress [6% × $18.00]	$1.08

= Expected per share $182.48

FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT

+5y	+10y	+15y	+20y
$280.77	$432.00	$664.68	$1,022.69
2.12×	3.26×	5.01×	7.71×

ULTRA BULL $370.13

ASSUMPTIONS

TAM Year-10 ($B)	25
Mature share (%)	22.4
Mature op margin (%)	40
Sales-to-capital	1.7
Terminal WACC (%)	8.5
Terminal growth (%)	3.0
P(failure) (%)	3
Scenario probability (%)	5

10-YEAR DCF · $B

FY	Rev	Margin	FCF	PV FCF
FY27	0.22	-60%	-0.17	-0.16
FY28	0.40	-10%	-0.15	-0.12
FY29	0.70	+15%	-0.07	-0.05
FY30	1.20	+27%	+0.03	+0.02
FY31	1.95	+33%	+0.20	+0.12
FY32	2.85	+37%	+0.53	+0.29
FY33	3.70	+39%	+0.94	+0.47
FY34	4.50	+40%	+1.33	+0.60
FY35	5.10	+40%	+1.69	+0.71
FY36	5.60	+40%	+1.95	+0.75

Σ PV explicit FCF	**+2.63**
+ PV terminal (g 3.0%)	14.06

EQUITY BUILD · $B & PER SHARE

Operating EV	$16.69B
+ Cash & investments	$0.82B
= Equity value	**$17.51B**
Raised over period	$0.20B
Dilution by FY36	+5%
FY36 shares (M)	46
DCF per share	**$380.65**
× (1−P_fail) [97%]	$369.23
+ P_fail × distress [3% × $30.00]	$0.90

= Expected per share $370.13

FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT

+5y	+10y	+15y	+20y
$556.55	$836.86	$1,258.35	$1,892.12
4.20×	6.31×	9.49×	14.27×

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GRAIL People · Opportunity · Context · Deal

Bear $12.82 Base $58.34 Bull $182.48

Underwrite the position the way a venture investor underwrites a round — across all four legs, public or private. **People** is the leg scored here (observable only); Opportunity, Context and Deal cross-reference the rest of the memo.

PEOPLE · WHO RUNS / OWNS / FUNDS IT

Josh Ofman — 0 yrs in seat

1.8%
insider ownership

MEDIUM
key-person risk

Capital allocation (realized)
GRAIL is pre-profit and cash-burning. It was spun out of Illumina in June 2024 (Illumina had paid ~$8B in 2021; the public market now values GRAIL near ~$4.8B — heavy value destruction under prior ownership, pre-spin). Current management has roughly halved the cash burn to ~$300M/yr and holds ~$823M cash with no debt, funding the company into the ~2028 Medicare window without an imminent raise. FY25 revenue $147M (+17%). No dividend or buyback — appropriate for a pre-scale development-stage screener.

Incentive alignment
CEO pay is predominantly equity (RSU grants per Form 4 filings); Ofman directly owns ~0.4M shares (~1%). Total insider ownership is low (~1.8%) — typical of a corporate spinco and weaker skin-in-the-game than a founder-led name, with no founder block on the register.

GOVERNANCE FLAGS

- single class of common stock, one vote per share — no dual-class or super-voting
- independent Chair (Greg Summe) is separate from the CEO — the Chair and CEO roles are split
- brand-new CEO: Josh Ofman became CEO 1 June 2026 (Bob Ragusa retired); ~7 years at GRAIL prior (CMO/President since 2019), but under a month in the top seat during the post-NHS-miss period
- pending securities class-action litigation following the NHS-Galleri primary-endpoint miss (lead-plaintiff deadline Aug 2026)
- Illumina retained ~14.5% at the June-2024 spin (a diminishing overhang as it divests); the franchise carries the history of ~$8B destroyed under Illumina ownership in 2021-2024 (pre-spin)

THE FOUR LEGS

People — observable composite · 1–5 ●●●○○

Opportunity
The scenario distribution · the business snapshot

Context
The 7 Powers analysis (Power Origination)

Deal
Open-market common — entry price weighed against the probability-weighted fair value, then position-sized.
-50.2% vs spot $132.60

PEOPLE READ

Professional management on a clean single-class register with an independent Chair (Summe) split from the CEO — but a brand-new CEO (Ofman, June 2026), thin insider ownership (~1.8%, no founder block), a fresh post-NHS securities class action, and the spinco history of ~$8B destroyed under Illumina hold it at a middling 3; the offsets are the split Chair/CEO, the disciplined ~$823M cash runway, and the absence of dual-class.

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GRAIL supporting analysis · disclaimers · glossary

Bear $12.82 Base $58.34 Bull $182.48

PUSHBACK · WHY THE BASE CASE IS TOO HARSH

1 **Only randomized MCED dataset.**
The NHS-Galleri RCT (~140k) is the largest, only randomized MCED dataset — a cornered resource even after the primary-endpoint miss (stage IV fell -14%; mortality follow-up extended).

2 **FDA filing rests on PATHFINDER 2, not NHS.**
The PMA is built on the positive PATHFINDER 2 (69.8% sensitivity for the 12 deadliest, 99.6% specificity) plus the NHS prevalent round — not the failed late-stage primary.

3 **Medicare law already signed.**
The MCED coverage benefit is law (Feb 2026); coverage is gated on FDA approval and earliest 2028 at ~$509 mt-stool-DNA parity, but the legislative risk is retired.

4 **Cash-funded into the coverage window.**
~$823M and burn halved to ~$300M/yr funds GRAIL into the ~2028 Medicare window without an imminent raise.

5 **Tiny share count amplifies the unlock.**
~43M shares means a reimbursed-volume inflection moves per-share value hard — the asymmetry cuts both ways.

FALSIFICATION TRIGGERS

Bull validation
FDA PMA approval (2026-27) · Medicare NCD activates (~2028) · broad private coverage · test volume sustains >40% growth

Bear validation
PMA rejected/delayed · Medicare narrow/post-2028 or stuck at ~$509 parity · Abbott-backed Cancerguard scales · private payers demand mortality data

Reframe needed
if the extended NHS follow-up shows a mortality benefit, the clinical-utility debate resolves and the bull/ultra weights rise materially

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GLOSSARY · CONCEPTS REFERENCED IN THE NARRATIVE

Galleri — GRAIL's blood test screening for 50+ cancers from one draw; sold self-pay (~$949) today as an LDT, FDA PMA filed Jan 2026 (accepted).

MCED — Multi-cancer early detection — blood-based screening across many cancer types; a new category GRAIL pioneered.

NHS-Galleri — The ~140k-participant UK randomized trial; missed its stage-shift primary endpoint (Feb 2026) but cut stage IV; mortality follow-up extended.

PMA / NCD — FDA Premarket Approval (filed Jan 2026, now accepted) and Medicare National Coverage Determination (earliest 2028, ~$509 mt-stool-DNA parity through 2030) — the two gates to reimbursed scale.

Sales-to-capital (s2c) — Damodaran reinvestment efficiency — incremental revenue per dollar of reinvested capital.